

WEIGHT LOSS

Tirzepatide Protocol

Dual GIP/GLP-1 therapy: titration, cross-titration from semaglutide, and monitoring

DOCUMENT WL-002	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO Prescribing clinicians and clinical staff	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

Tirzepatide acts on both GIP and GLP-1 receptors and has its own titration ladder, tolerance profile and maximum dose. It is not interchangeable with semaglutide on a milligram basis and must never be substituted or converted arithmetically.

ABSOLUTE CONTRAINDICATIONS

Personal or family history of medullary thyroid carcinoma, or MEN2. Known hypersensitivity. Identical screening to semaglutide — ask explicitly and document the answer.

Candidacy, baseline evaluation, storage, cold chain, administration technique and documentation requirements follow WL-001 unless stated otherwise below. This protocol covers what differs.

2 Titration Schedule

Verify against current manufacturer labeling before adoption. Escalation is slower and longer than semaglutide, with more steps.

STANDARD WEIGHT-MANAGEMENT ESCALATION

Phase	Weekly dose	Minimum duration	Notes
Initiation	2.5 mg	4 weeks	Tolerance-building dose only.
Escalation 1	5 mg	4 weeks	First therapeutic dose. Many patients do well here.
Escalation 2	7.5 mg	4 weeks	Advance only for inadequate response with good tolerance.
Escalation 3	10 mg	4 weeks	Reassess whether further escalation is warranted.
Escalation 4	12.5 mg	4 weeks	Second-line maintenance option.
Maximum	15 mg	Ongoing	Maximum labeled dose. Do not exceed.

MAINTENANCE IS NOT ALWAYS THE MAXIMUM

A patient meeting their goal comfortably at 7.5 mg does not need 15 mg. The maximum dose is a ceiling, not a destination. Record the reasoning for the maintenance dose chosen.

3 Cross-Titration from Semaglutide

NO DOSE CONVERSION EXISTS

There is no validated milligram-to-milligram equivalence between semaglutide and tirzepatide. Converting 2.4 mg semaglutide into a 'comparable' tirzepatide dose is not supported and risks significant adverse effects. Switch by starting the tirzepatide ladder at an appropriate initiation dose.

- 1 **Document the reason for switching** — inadequate response, intolerance, supply, or patient preference. This is a clinical decision, not an upgrade.
- 2 **Stop semaglutide.** Take the accumulated effect of its long half-life into account when timing the first tirzepatide dose; your medical director sets the practice interval.
- 3 **Start tirzepatide at an initiation dose** appropriate to the patient's tolerance history, as determined by the prescriber. Do not start at a high dose because the patient 'tolerated semaglutide fine.'
- 4 **Re-titrate on the standard schedule** with the same minimum intervals.
- 5 **Monitor closely through the first two escalations** — overlapping residual effect plus a new agent is the highest-risk window for GI adverse effects.
- 6 **Record the full rationale**, the interval used, and the starting dose chosen.

4 Response & Plateau

Set expectations at intake, not at the plateau. Weight loss is not linear, it slows over time, and a plateau is a physiological adaptation rather than a treatment failure.

Situation	Assessment	Action
Steady loss, good tolerance	Therapy working as intended.	Hold current dose. Do not escalate purely to accelerate.
Slowing rate after several months	Expected adaptation.	Reinforce nutrition and resistance training per WL-007. Reassess goal. Escalate only if criteria in WL-004 are met.
Genuine plateau at submaximal dose with good tolerance	Inadequate response.	Consider escalation per WL-004 after confirming adherence and reviewing intake.
Plateau at maximum dose	Maximum benefit from this agent reached.	Discuss maintenance, alternative agents, or referral. Do not exceed the labeled maximum.
Weight loss with poor oral intake or lean mass loss	Adverse, not successful.	Address intake and protein per WL-007. Consider dose reduction.

5 Monitoring & Red Flags

- Assessment at every visit as per WL-001, including resting heart rate.

- Laboratory monitoring per WL-009.
- GI symptom grading per WL-005 at every contact.
- Hypoglycemia risk is meaningful only in combination with insulin or a sulfonylurea — identify those patients and coordinate with their prescriber before initiation.
- Gallbladder symptoms, particularly during rapid weight loss.
- Injection site reactions, which are more commonly reported with this agent.

SAME STOP CRITERIA AS WL-001

Severe abdominal pain radiating to the back with vomiting, persistent vomiting with dehydration, signs of bowel obstruction, or a suspected allergic reaction — hold the drug and arrange same-day medical evaluation.

ANESTHESIA WARNING APPLIES EQUALLY

Delayed gastric emptying and aspiration risk under sedation apply to tirzepatide as much as to semaglutide. Give the instruction, and record that you gave it.

6 Quick Reference

QUICK REFERENCE CARD

Tirzepatide essentials

1. **Ladder:** 2.5 → 5 → 7.5 → 10 → 12.5 → 15 mg. Four weeks minimum per step.
2. **Maximum is a ceiling, not a target.** Document the maintenance dose chosen.
3. **No conversion from semaglutide.** Restart the ladder.
4. **Highest-risk window** is the first two steps after a switch.
5. **Insulin or sulfonylurea on board?** Coordinate before initiating.
6. **Same stop criteria** as WL-001. Same anesthesia warning.
7. **Confirm mg, then convert to volume.** Never dose in syringe units.

Print, laminate and post at the point of care

7 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME	LOCATION / SITE
MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS	LICENSE NUMBER & STATE
SIGNATURE	DATE ADOPTED / NEXT REVIEW DATE

Template notice. This document is an editable template provided for educational and operational guidance. It is not legal advice, does not constitute a physician–patient relationship, and does not guarantee regulatory compliance or the outcome of any inspection. Drug doses, concentrations and device settings must be verified against current manufacturer labeling and prevailing clinical guidance before use. A licensed medical director must review, customize and sign this protocol before it is placed into service, and must confirm that every task assigned within it falls inside the scope of practice of the personnel performing it in your state.

© MedSpa Standards. Licensed for use within the purchasing practice only. Redistribution or resale is prohibited.